

Patient Name: 
Patient Number: 

Date: 
Date: 

INITIAL ONCOLOGY CONSULT

Primary Diagnosis:
Malignant neoplasm of pancreatic duct

Date	Type	ICD-9	ICD-10	Description	Disease Status	Status Date
9/29/2025	Primary	157.3	C25.3	Malignant neoplasm of pancreatic duct		

Other Medical Problems:

Date	Type	ICD-9	ICD-10	Problem	Comment	Status
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Reason for Today's Visit:
9/29/2025: 


History of Present Illness


s. CA 19-9 was 215, repeat 171. On 09/24/2025, patient underwent ERCP with biliary stent and obtained biopsies. Biopsy results confirmed adenocarcinoma of the pancreatic head as well as positive brushing from the bile duct. Patient was discharged from the hospital last Thursday with improved jaundice and bilirubin decreasing from a peak of 14-5.1 at discharge.

Patient reports significant dorsal pain that began approximately one week before his jaundice diagnosis. Pain primarily affects his lower back and radiates to his flanks. Pain is most severe at night and interferes with sleep, causing patient to move to the couch after about 45 minutes in bed. Heat does not provide relief. Patient has used ibuprofen with some symptomatic improvement. He denies anterior abdominal pain but reports occasional discomfort when eating.

Patient notes recent weight loss of approximately 5 lbs since hospital discharge and reports decreased appetite. His urine color has improved from the dark color noted during his jaundiced state and his stools are less pale in color.

Past Surgical History
- Total thyroidectomy 3 years ago for large nodule compressing trachea
- Hernia repair at age 9



PMH

- Pancreatic ductal adenocarcinoma, newly diagnosed
- Type 2 diabetes mellitus
- Essential hypertension
- Hyperlipidemia
- Post-procedural hypothyroidism, status post total thyroidectomy 3 years ago for large nodule

Social History

- Occupation: Data analytics, working from home
- Denies smoking history, reports only secondhand smoke exposure from grandparents
- No alcohol history reported
Patient is married.
Patient lives with spouse.
Non smoker Not applicable Denies any prior alcohol use.
Denies any illicit drug use.
Data Analytics

Family History

- Mother: Lung cancer, deceased (elderly at time of death)
- Father: Alive, residing in nursing home

Personal History

Patient denies having ever received blood products in the past.

Allergies

No Known Drug Allergies

Medications

Inside	Drug	Script Date	Qty	Rfls	Instructions
	AMLODIPINE BESYLATE 5 MG TAB	7/24/2025	90	0	TAKE 1 TABLET EVERY DAY BY ORAL ROUTE IN THE MORNING.
	LEVOTHYROXINE 100 MCG TABLET	7/24/2025	90	0	TAKE 1 TABLET BY MOUTH EVERY DAY
Y	loperamide 2 mg capsule	9/29/2025	180	1	Initial: 4 mg, followed by 2 mg every 2 to 4 hours or after each loose stool as needed for chemo induced diarrhea
	METFORMIN TAB 500MG	1/1/1753	60	0	
	metformin 500 mg tablet	9/29/2025		0	2 tablets daily
Y	ondansetron 4 mg disintegrating tablet	9/29/2025	120	1	Take 1 tablet every 6-8 hrs as needed for chemo induced n/v
	ROSUVASTATIN CALCIUM 20 MG TAB	7/24/2025	90	0	TAKE 1 TABLET BY MOUTH EVERY DAY

Review Of Systems

Constitutional: Positive for weight loss (5 lbs), positive for decreased appetite. No fever.
HEENT: +scleral icterus.
Gastrointestinal: No nausea, no vomiting. Positive for occasional discomfort with eating.

Musculoskeletal: Positive for severe lower back pain radiating to flanks, worse at night, not responsive to heat application.
Skin: No jaundice (improved).
Urinary: No dark urine (improved).

Vitals

Vitals on 9/29/2025 1:42:00 PM: Height=67in, Weight=184.3lb, Temp=97.2f, Pulse=85, SystolicBP=123, DiastolicBP=94, O2 Sat=97%

Physical Exam

GENERAL: In no acute distress.
SKIN: No rash noted. No lesions. + Jaundice
EYES: Sclera clear, no jaundice noted. Conjunctiva clear.
CARDIOVASCULAR: Regular rate and rhythm.
CHEST/RESPIRATORY: No dyspnea. No rhonchi. No wheezing. No decreased breath sounds at bases.
ABDOMEN: Soft. Nontender. Non distended
HEMATOLOGIC/LYMPHATIC: No petechiae. No purpura.
MUSCULOSKELETAL: No spinal tenderness
NEUROLOGIC: Affect normal.
PSYCHIATRY: Oriented to person/place/time.
EXTREMITIES: No edema. No calf tenderness

Patient reported pain

Pain Scale 0 - No pain

Treatment Recommendations:

Reassess pain at next visit. Continue current pain regimen Refer to palliative care specialist

ECOG

Performance Status -ECOG 0: Fully active, able to carry on all pre-disease performance without restriction

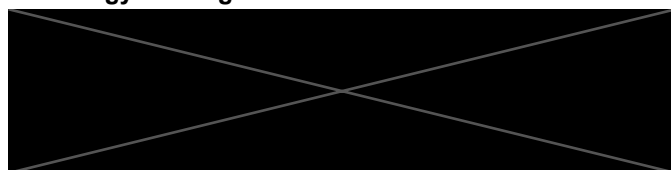
Labs

- Hemoglobin: 12.5 g/dL (reference range 13-17), mild anemia

Labs

Lab results on 9/29/2025: WBC=8.7 x10³/UL, RBC=**4.31** x10⁶/UL, Hgb=**12.5** g/dl, HCT=**38.1** %, MCV=88.4 fL, MCH=29.0 pg, MCHC=32.8 g/dL, RDW [Ratio]=14.2 %, MPV=10.1 fL, Plat=275 x10³/UL, ANC=6.0 x10³/UL, Lymph#=1.5 x10³/UL, MONO#=0.8 x10³/UL, BASO#=0.1 x10³/UL, EOS#=0.2 x10³/UL, Gran%=69.7 %, Lymph%=17.6 %, MONO%=9.2 %, BASO%=0.8 %, EOS%=1.8 %

Radiology Findings



ream main pancreatic ductal dilatation and parenchymal atrophy, encasement of SMA and portal vein with moderate narrowing of SMA. aluation recommended.

ion with abrupt tapering at the distal CBD in the region of the pancreatic head/neck, likely secondary to pancreatic mass. No intraductal enhancement or intraductal mass.
3. Mild interstitial pancreatitis at the pancreatico duodenal groove with mild associated edema.
4. Mild splenomegaly.

CT a/p 9/22/25

IMPRESSION: 1. Lobulated fullness of the pancreatic head with a discrete area of hypodensity measuring approximately 1 cm in maximal width. Dilatation of the pancreatic duct to 0.9 cm. Diffuse intrahepatic biliary dilatation. Constellation of findings is suspicious for a pancreatic head mass. Further characterization with endoscopic ultrasound is recommended.
2. Contracted gallbladder with stones.
3. Sigmoid diverticulosis without evidence of diverticulitis.
4. 2 cm left upper pole renal cyst.

s/p ERCP with sphincterotomy and stent placement on 9/24/25

all.

- A biliary tract obstruction secondary to what appeared to be a mass was found in the common bile duct.
- A biliary sphincterotomy was performed.
- Cells for cytology obtained in the lower third of the main duct.
- One plastic stent was placed into the common bile duct.

s/p EGD - Normal esophagus. - Gastritis. - Normal examined duodenum. - No specimens collected.

s/p EUS with FNA

- A mass was identified in the uncinate process of the pancreas. Fine needle biopsy performed.
- One enlarged lymph node was visualized in the peripancreatic region. Fine needle biopsy performed.
- There was dilation in the common bile duct which measured up to 20 mm.
- There was no evidence of significant pathology in the visualized portion of the liver.

CT chest 9/26/25

IMPRESSION: 1. No acute CT pathology. Scattered calcified granulomas.

2. Moderately dilated 4.9 cm sinus of Valsalva, similar to prior.

3. Moderate splenomegaly.

4. Pancreatic ductal dilatation with abrupt transition in the ampullary region, CBD stent graft. Cholelithiasis/calcifications within the CBD. Correlation with MRI abdomen report from 09/20/2025 is recommended.

5. Adenomatous thickening left adrenal gland.

6. Elevated right hemidiaphragm.

Radiology Reports

Print? ☐	Date of Doc.	Name	MD Interpretation	Comment
☐	9/20/2025	MRI ABD		
☐	9/21/2025	CHS - X RAY CHEST		
☐	9/21/2025	CHS - US ABDOMEN COMPLETE		
☐	9/22/2025	CHS - CT ABDOMEN AND PELVIS		
☐	9/26/2025	CT CHEST		

Pathology Reports

Non-Gynecologic Cytology Report Diagnosis

A. Pancreatic mass, endoscopic ultrasound-guided aspiration and cell block:

- Positive for malignant cells.
- Consistent with adenocarcinoma.

B. Retroperitoneal lymph node, endoscopic ultrasound-guided aspiration and cell block:

- Atypical cells present.
- Rare atypical cells in a background of sparse lymphoid cells (see comment).

C. Biliary, brushings:

- Positive for malignant cells.
- Consistent with adenocarcinoma.

D. Bile, cytology with cell block:

- Suspicious for malignant cells.
- Extremely scantily cellular specimen containing rare cells suspicious for adenocarcinoma

. Comment The specimen in part A and C contains cells consistent with adenocarcinoma. The retroperitoneal lymph node biopsy in part B is scant but contains lymphoid cells. Rare poorly preserved atypical cells are seen not quite resembling those in part A.

Print? ☐	Date of Doc.	Name	MD Interpretation	Comment
☐	9/24/2025	PANCREATIC MASS		

Assessment:

██████████ with newly diagnosed locally advanced pancreatic ductal adenocarcinoma presenting for initial oncology consultation.

Patient initially developed dorsal pain approximately one week prior to developing jaundice. After presenting to his PCP with jaundice, an MRI revealed a 1.3 cm uncinete process mass with upstream main pancreatic ductal dilatation and parenchymal atrophy, findings consistent with pancreatic adenocarcinoma. Partial encasement of SMA and portal vein with moderate narrowing of SMA. Mild portal venous narrowing causing intrahepatic and extrahepatic biliary ductal dilatation with abrupt tapering at the distal CBD in the region of the pancreatic head/neck, likely secondary to pancreatic mass. No intraductal enhancement or intraductal mass. CT abdomen/pelvis on 09/22/2025 confirmed these findings. CA 19-9 215. repeat 171. Patient underwent ERCP with stent placement and biopsy on 09/24/2025, which confirmed adenocarcinoma of both the pancreatic mass and bile duct brushings. CT chest with no evidence of metastatic disease.

The tumor is locally advanced and currently unresectable due to arterial invasion. Patient's bilirubin has improved from a peak of 14-5.1 at hospital discharge, with clinical improvement in jaundice. Patient is experiencing significant dorsal pain, likely tumor-related, that interferes with sleep.

Recommendation/Plan:**Pancreatic Ductal Adenocarcinoma**

- Classified as locally advanced due to arterial invasion, making patient currently inoperable
- Will start neoadjuvant FOLFIRINOX chemotherapy (5-FU, oxaliplatin, irinotecan) with treatment given every 14 days, orders placed today.
- Orders placed for chemo port placement at New Hyde Park facility to be scheduled as soon as possible
- Will aim to start first treatment cycle ASAP
- Will administer Neulasta 24 hours after chemotherapy to support white blood cell count
- Will perform CT after 2-3 cycles to assess response
- May consider additional 3 cycles depending on response
- Goal is tumor shrinkage to potentially make it resectable
- Will send tissue for molecular testing (Caris). Sent in vitae genetic testing
- Will maintain communication with surgeon (Dr. Ganesh) regarding treatment progress
- Offered a 2nd opinion with MSK to discuss potential enrollment in a clinical trial, but patient declined.

Pain Management

- May continue ibuprofen with food as needed for pain control
- Referred to palliative care for comprehensive pain management
- Counseled that pain may improve with tumor response to treatment
- Offered opioids for better pain control but pt declined at this time.

Endocrine Management

- Referred to Dr. Rattan for diabetes management during chemotherapy

Anemia

- Will monitor hemoglobin levels closely during treatment
- Current mild anemia may worsen with chemotherapy
- anemia workup sent

Symptom Management

- Prescribed ondansetron for chemotherapy-related nausea/vomiting
- Prescribed loperamide for potential irinotecan-induced diarrhea
- Educated regarding potential cold sensitivity with oxaliplatin
- Advised regarding risk of peripheral neuropathy with treatment
- Advised to report fever $\geq 100.4^{\circ}\text{F}$ immediately as this may indicate infection in setting of chemotherapy-induced neutropenia
- Counseled regarding potential hair loss, fatigue, and mouth sores

PLAN SUMMARY

- Chemo port placement to be scheduled as soon as possible
- Start FOLFIRINOX chemotherapy Monday 10/6/2025
- Pre-medications will be given before chemotherapy administration
- 5-FU will be administered via continuous infusion pump for 48 hours
- Neulasta on-body injector to be applied 24 hours after treatment
- Prescriptions provided for antiemetics and antidiarrheals
- Patient to wear mask in public to reduce infection risk
- - cycles

- Continue coordination with surgical team

Treatment Plan:

Chemotherapy.

Diagnosis Plan:

Order Tests: *CBC/Diff/Platelets, *CEA, *CA-19.9, *Comp Metabolic Profile (CMP), *Magnesium, *LDH, *Uric Acid, *Phosphorus, *Haptoglobin, *FE/Iron, *Ferritin, *Total Iron Binding (TIBC)/UIBC, *Retic Count, *B-12, *Folic Acid / Folate, InVitaie Gentic Testing Ordered/MT, Caris Molecular Profiling Submitted. Order Activities: MD-New PT- Oncology, In-house Consult Request: Interventional Radiology, In-house Consult Request: Palliative Care, External Consult Request: Endocrinologist, In-house Consult Request: Endocrinology Order Chemo: . The goal of treatment is to improve disease-free survival. This was highlighted in our discussion. Risks and benefits of therapy were presented and discussed with the patient and their family/spouse. All questions and concerns were addressed to patient's satisfaction and understanding was expressed. Side-effects were reviewed and appropriate measures of support will be provided. Re-staging scan will be set up for assessing disease status and response to treatment. We had discussed the composition of chemotherapy regimen, length of chemo-cycle, duration of treatment and the time to assess response to treatment. The need for a Mediport placement and alternative options were discussed. A referral will be made for the procedure.

Supportive Care Measures Plan:

Supportive care measures are necessary for patient well-being and will be provided as necessary.

Documents & Plan Reviewed

Patient's medical records were reviewed Patients laboratory results were reviewed and explained in full. All questions answered Pathology results reviewed and explained in full Radiology imaging and dictated report were reviewed; the disease status and level of response to the treatment were explained in great detail NCCN guidelines were reviewed and shared with the patient

Care Coordination & Patient Education

Obtain patient medical records. More than half of time was spent for providing counseling and coordination of care. We spent sufficient time to discuss many aspects of care; questions were answered to patient's satisfaction. The diagnosis and care plan were discussed with patient in detail. Chemotherapy education was provided and continuously reinforced throughout our discussion. Performance status was assessed. Preparation for chemotherapy, including laboratory tests was arranged and available lab results were reviewed. Alternative options were discussed and pros and cons of each option were illustrated. Risk and benefit of the treatment were discussed in detail.



Schedule follow-up in:

2 weeks.

Attestation

"AI"generated content.

Portions of this note were generated with an ambient AI scribe.



Signed

